

Person Centered Support Plan Approved ⓘ

This Individual Plan is generated from Tennessee Department of Disability & Aging

Individual	BARBARA C PHAGAN	Photo
Oversight ID	0000010468 (DIDD-TN)	
Date of Birth	01/22/1947	
Medicaid Number	TD510568555	
Edition Type	Annual	
Meeting Date	11/18/2025	
Start Date	01/17/2026	
End Date	01/16/2027	

About Me

Jump to ?

What People Admire about Me

Barbara is a very loveable, friendly and affectionate person. She loves to give hugs.
Over time, Barbara will remember specific details about you and your life. She loves to ask about others children and family member.
Barbara is very girly. She loves to have on jewelry and a purse from her collection.
Majority of the time, Barbara is always smiling, loves to laugh and is very social
Barbara likes to joke with people
Barbara is very helpful and enjoys helping others

?

What is Important to Me

Home: Dressing and looking pretty is important to Barbara. She loves to be complimented by her staff and others. Barbara was very close with her housemates. They had lived together since they moved from Clover Bottom.
Barbara still lives with one of those roommates. It is important to Barbara to have her many stuffed animals in her bed room. She also does not want others

?

to invade her personal space. It is important to Barbara that she is able to rest and relax in her home. It is very important to Barbara that she be able to spend time talking to others, going shopping, and going out to eat.

Community: Barbara enjoys being involved in her community and having the opportunity to interact with others is very important to Barbara. In the past, Barbara enjoyed going to the Discovery Center and playing with the children. Barbara does like to go out-even if she just wants to go for a ride and to get some tea. Sometimes Barbara is very tired and would like to be at home. Barbara is sometimes tired and wants to come home or take a nap. Barbara must be able to relax when she needs to. When the seasons change, Barbara may be lethargic and not want to do much.

**How to
Support Me
Best**

Supports in the Home: Barbara currently lives in a single-family home located in LaVergne TN. She has one male housemates, who she has lived with for many years. They get along very well. Barbara's home contain modifications that easily adapt living spaces to meets her safety needs such as holding bars located in the bathrooms and wheel-chair accessible ramps at the entrance/exit doors. In the event of a life-threatening situation occurring in the home, she will be unable to recognize any dangerous situations/areas. Therefore, she would need total assistance to seek safety from a dangerous situation. She needs continuous prompting from staff to remind to use her hearing aids correctly and to place them in a safe place. She also uses a 2 L oxygen concentrator when as needed that was prescribed by her licensed physician. Therefore, staff must be properly trained to make adjustments under the discretion of her licensed physician. During Barbara's bedtime, staff must perform routine bed checks must be conducted at 12:00 A.M., 4:00 A.M., & 7:00 A.M. every night to ensure that the flow of oxygen is operating at the correct level prescribed and ensure she is not experiencing difficulty breathing and to change her soiled briefs when needed.

As result of her age and balance problems, Barbara is experiencing more difficulty with walking without falling. According to the physical therapist, she has an unsteady gait that become worse on uneven surfaces or when surfaces change in texture and/or height. "She does not pick up her feet during gait patterns but mildly slides them on the floor. She also requires hand held assistance or close stand by assistance with ambulation in the home. Significant limitations with ambulation and balance skills when ambulating outside the home contributing to difficulty with her vision and ability to discern terrain changes." Staff have witnessed a decline in Barbara's physical mobility and has had an increase in falls. Barbara's vision is rapidly declining which pose a greater fall risk. Therefore, it is important for staff have a line of sight of Barbara to assess for fall prevention.

Supports in the Community: Barbara receives community participation and intermittent wrap around services. Similar to the support in the home, she shares the support needs in the community such assistance with ambulation, assistance



to recognize dangerous situations/areas, oxygen therapy, etc. When she is feeling tired from community participation, she needs the use of her wheelchair and needs frequent rest breaks. When Barbara is dining out, staff would need to ensure that she follows her dietary meal plan. When she is presented with several options of community activities, she will verbalize her choice of different activities and return back home to rest, perform personal hygiene, or choose other leisure activities. When making purchases, she needs staff assistance to count money for correct change in return. Sometimes, she tends to kiss and hug random individuals, so staff have to redirect her displaying inappropriate social behaviors.

ADLs & Household Chores: Previously, she was able to perform certain ADLs related to personal hygiene and grooming and certain household chores such as sorting laundry and removing dirty dishes from the table. Now, she needs total assistance from staff due to her worsening hand tremors, poor body balance, and decreasing cognitive ability. When showering and/or toileting, its important for Barbara to use the safety equipment such as the shower bench/chair, grab bars, raised toilet seat and staff's hand for balance. When she is dressing and/or undressing, staff must be within an arm's reach to help for balance. In performing oral hygiene, staff must assist with the cleansing of her dentures and lower bridge to prevent choking. Again, all performance of ADLs and household chores requires total staff assistance. She cannot perform these activities alone regardless of the circumstances.

When prompted from staff, Barbara will attempt to perform certain household chores such as sorting out laundry but however she needs staff assistance with coordinating colors to match and ensure the clothing are appropriate for the weather. Again, Barbara needs assistance with all household chores such as laundry, cleaning her room, dusting, and vacuuming, etc.. She requires both verbal and physical prompts to complete certain activities. Barbara often gets confused when doing her laundry. She needs help with separating her clothes by the colors. Barbara had an OT evaluation completed and plans developed for Home Exercise, Oral Hygiene and Showering/Shower Transfers. All plans are located in the Training Specifics book.

Physical Therapy Services: Prior, an assessment for PT services occurred in April 2022 and on-going services were recommended. When PT services were provided, it had addressed limitations with ambulation outside her home by addressing gait training utilizing a gait belt and assistive device to allow increased confidence with ambulation and balance skills. PT also provided staff instructions and training.

Enabling Technology & Medical Equipment: Barbara depends on several different safety devices such as cordless bed sensor pads and a cordless floor mat to alert staff for prevention of a fall or any other potential injures. When new hearing aids are purchased, then staff have to remind Barbara to properly use the hearing aids daily. She also uses a 5 L oxygen concentrator that was

prescribed by her licensed physician. Therefore, staff must perform routine bed checks must be conducted at 12:00 A.M., 4:00 A.M., & 7:00 A.M. every night to ensure that the flow of oxygen is operating at the correct level prescribed.

Clinical Nutrition Services: Barbara was referred to nutritional services due to PKU, GERD, constipation, and high cholesterol. Barbara's regularity and symptoms of GERD continues to improve. Although, she continues to use Phlexy-10, 5x per day, she continues to remain slightly under the recommended weight range of 110-137 lbs. As a result of the diagnosis of PKU, she would benefit from nutrition services to decrease risk of GERD complications and decrease cardiac risks associated with high cholesterol. RD also needs to train new staff on Barbara's specific diet plan. **The 2026 nutrition plan of care is in the process of being signed and will be provided to the ISC.**

Speech Therapy Services: Barbara was referred to speech therapy services to include assisting with activities to facilitate safe eating strategies, auditory comprehension of language and hearing aid usage and care. During the current service period, she lost her dental partial plate. The therapist modified Barbara's mealtime plan to include ground diet texture. As a result of Barbara's hand tremors growing worse, the mealtime plan was modified to include total assistance to ensure that she is maintaining her body weight. Barbara shown improvement in following one step instructions. Barbara's short term memory deficit requires staff to repeat instructions. SLP provides services at home, via teletherapy, and in the community.

Behavior Analyst Services: COS agreed to the discontinuation of behavioral analysis services because of her worsening cognitive impairment and previous positive progress with the services. She may display negative behaviors such as throwing personal items and display minor physical acts of aggression towards staff, but staff has been trained to redirect her from these behaviors from the previous BSP.

History of Hospitalizations: In October 2024, Barbara had twisted her ankle and was hospitalize for a UTI and Pneumonia. In September 2024, she was diagnosed with two UTIs. She was prescribed Macrobid 100mg daily x 14 days. She had a follow-up appointment with her PCP and received a specialty referral to a Urologist for further examination and treatment.

Previous Employment: Previously, Barbara had maintain employment working at a beauty outlet store. Now, she does not wants to obtain employment and she is exempt from the EIC process due to her age.

Consumer Direction: Barbara and the COS discussed Consumer Direction of services. Barbara and conservator decided that Barbara does not want to consumer direct services at this time. They understand they can change their minds at any time.

Waiver Enrollment: Barbara is enrolled in the Statewide Waiver and this works well for Barbara. Barbara, her conservator, and COS discussed what options Barbara would have if involuntary disenrollment from the Statewide Waiver were to occur. The COS agrees Barbara would be unable to live on her own, and family is unable to care for her at this time, so Barbara would be enrolled in Choices in the event of involuntary disenrollment. Barbara’s conservator and COS understand that if disenrolled from the Waiver, she would be unable to reenter.

Wraparound Service: Barbara sometimes comes home to eat lunch or to change clothing. She might also come home if she's not feeling well or if the weather becomes inappropriate/unsafe for Barbara to be out. Once home, Barbara might decide to go out later, or choose to stay in for the rest of the day/evening. Barbara's provider agency understands that wrap-around is not a stand-alone service and can only be used in conjunction with community participation services.

Legal Decision Makers

Jump to

?

Name	Residential Address	Mailing Address	Phone	Email	Relationship with the Individual
Melissa Redmond	1324 Lawnville Road, Kingston, TN 37763, USA	1324 Lawnville Road, Kingston, TN 37763, USA	615-522-2550	melissa.redmond@mdcconservator.org	Guardian

Risks

Jump to

?

Risk Type	Comments	Support Documents	Support Document Information			
			Home		Work	
			Date	Where Kept	Date	Where Kept
Increased or Unusual Falls	Barbara has had several falls and continues to at times say she is falling, even if she is seated.					
Inability to Tolerate a Medical Examination/Procedure	Barbara requires IV sedation requested for dental procedures and her PCP will be contacted if there is a need for PRN medications to help her deal with the anxiety surrounding medical procedures. If she requires general anesthesia, which per her conservator has been requested in the past- anesthesia would need to be in a hospital setting.					
Inability to Communicate Basic Needs such as Pain, Illness, Hunger or Thirst	Barbara may have difficulty expressing pain or illness and the cause. She may also indicate that she has pain or illness as an attention seeking behavior. Needs help communicating with hospital staff; would be unable to call for help if needed. Any claims of discomfort and pain by Barbara should be taken seriously. If Barbara exhibits confusion for doing basic tasks, such as forgetting how to drink from her cup, she should be taken to be evaluated by a medical professional as this is a sign something is very wrong.					

Risk Type	Comments	Support Documents	Support Document Information			
			Home		Work	
			Date	Where Kept	Date	Where Kept
Significant Weight Gain/Loss or Change in Eating Patterns	Hx of overweight and underweight body status; Barbara has been diagnosed with PKU; phenylalanine intakes controlled by diet are normally within acceptable range;					
Choking and/or Aspiration or Swallowing Disorders	Barbara is at increased risk of choking due to mealtime issues such as eating fast, talking while eating, losing food and fluids from mouth, and inadequate chewing due to use of dentures					
Elopement	Informed of risk of elopement					
Self Injury	In the past self-harm has been a risk, such as biting herself					
History of or Presently Engages in Aggressive or Dangerous Behavior	Showed forms of physical aggression in the past					

Professional Services

[Jump to](#)


Nothing found to display

Nursing

[Jump to](#)


Has a Nursing Care Plan at Home?	No	Where Found
Has a Nursing Care Plan at Work?	No	Where Found
Has a Health/Medical Problem List?	No	Where Found (Home)
		Where Found (Work)
Has a Health Care Representative?	No	Appointed By
Appointment Date		Where is the Document Located?

Service Supports

Jump to?

Filter

Region	Funding Source	Service(s)	Status	Service Provider	Service Date From	Service Date To
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	T2022 - INDEPENDENT SUPPORT COORDINATION (T2022) (Billable)	Approved	BGCINC-TN-MIDDLE - BGC INC - MIDDLE	02/01/2026	06/30/2026
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	T2022 - INDEPENDENT SUPPORT COORDINATION (T2022) (Billable)	Approved	BGCINC-TN-MIDDLE - BGC INC - MIDDLE	07/01/2026	12/31/2026

Region	Funding Source	Service(s)	Status	Service Provider	Service Date From	Service Date To
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	T2022 - INDEPENDENT SUPPORT COORDINATION (T2022) (Billable)	Approved	BGCINC-TN-MIDDLE - BGC INC - MIDDLE	01/01/2027	01/16/2027
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	T2021-U2 - COMMUNITY PARTICIPATION SUPPORTS (LEVEL 4) (T2021 - U2) (Billable)	Approved	KHARIS-TN-MIDDLE - KHARIS CARE LLC-MIDDLE	01/17/2026	06/30/2026
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	T2021-U2 - COMMUNITY PARTICIPATION SUPPORTS (LEVEL 4) (T2021 - U2) (Billable)	Approved	KHARIS-TN-MIDDLE - KHARIS CARE LLC-MIDDLE	07/01/2026	12/31/2026
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	T2021-U2 - COMMUNITY PARTICIPATION SUPPORTS (LEVEL 4) (T2021 - U2) (Billable)	Approved	KHARIS-TN-MIDDLE - KHARIS CARE LLC-MIDDLE	01/01/2027	01/16/2027
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	T2021-U5 - INTERMITTENT EMP & COMM PART WRAPAROUND (LEVEL 4) (T2021 - U5) (Billable)	Approved	KHARIS-TN-MIDDLE - KHARIS CARE LLC-MIDDLE	01/17/2026	06/30/2026

Region	Funding Source	Service(s)	Status	Service Provider	Service Date From	Service Date To
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	T2021-U5 - INTERMITTENT EMP & COMM PART WRAPAROUND (LEVEL 4) (T2021 - U5) (Billable)	Approved	KHARIS-TN-MIDDLE - KHARIS CARE LLC-MIDDLE	07/01/2026	12/31/2026
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	T2021-U5 - INTERMITTENT EMP & COMM PART WRAPAROUND (LEVEL 4) (T2021 - U5) (Billable)	Approved	KHARIS-TN-MIDDLE - KHARIS CARE LLC-MIDDLE	01/01/2027	01/16/2027
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	T2033-U4-UN - SUPPORTED LIVING LEVEL 4 - 2 PEOPLE (T2033 - U4 - UN) (Billable)	Approved	KHARIS-TN-MIDDLE - KHARIS CARE LLC-MIDDLE	01/17/2026	06/30/2026
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	T2033-U4-UN - SUPPORTED LIVING LEVEL 4 - 2 PEOPLE (T2033 - U4 - UN) (Billable)	Approved	KHARIS-TN-MIDDLE - KHARIS CARE LLC-MIDDLE	07/01/2026	12/31/2026
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	T2033-U4-UN - SUPPORTED LIVING LEVEL 4 - 2 PEOPLE (T2033 - U4 - UN) (Billable)	Approved	KHARIS-TN-MIDDLE - KHARIS CARE LLC-MIDDLE	01/01/2027	01/16/2027
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	S9470-U1-TS - NUTRITION 1 (S9470 - U1 - TS) (Billable)	Approved	GREGORY-TN-MIDDLE - MARY EVA GREGORY - MIDDLE	02/01/2026	02/28/2026

Region	Funding Source	Service(s)	Status	Service Provider	Service Date From	Service Date To
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	S9470-U1-TS - NUTRITION 1 (S9470 - U1 - TS) (Billable)	Approved	GREGORY-TN-MIDDLE - MARY EVA GREGORY - MIDDLE	04/01/2026	04/30/2026
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	S9470-U1-TS - NUTRITION 1 (S9470 - U1 - TS) (Billable)	Approved	GREGORY-TN-MIDDLE - MARY EVA GREGORY - MIDDLE	06/01/2026	06/30/2026
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	S9470-U1-TS - NUTRITION 1 (S9470 - U1 - TS) (Billable)	Approved	GREGORY-TN-MIDDLE - MARY EVA GREGORY - MIDDLE	08/01/2026	08/31/2026
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	S9470-U1-TS - NUTRITION 1 (S9470 - U1 - TS) (Billable)	Approved	GREGORY-TN-MIDDLE - MARY EVA GREGORY - MIDDLE	10/01/2026	10/31/2026
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	S9470-U1-TS - NUTRITION 1 (S9470 - U1 - TS) (Billable)	Approved	GREGORY-TN-MIDDLE - MARY EVA GREGORY - MIDDLE	12/01/2026	12/31/2026
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	G0153-U4-GN - SPEECH LANG HEARING 1 (G0153 - U4 - GN) (Billable)	Approved	SPPS-TN-MIDDLE - SPEECH PATHOLOGY SPECIALIST, LLC-MIDDLE	02/01/2026	02/28/2026
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	G0153-U4-GN - SPEECH LANG HEARING 1 (G0153 - U4 - GN) (Billable)	Approved	SPPS-TN-MIDDLE - SPEECH PATHOLOGY SPECIALIST, LLC-MIDDLE	03/01/2026	03/31/2026

Region	Funding Source	Service(s)	Status	Service Provider	Service Date From	Service Date To
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	G0153-U4-GN - SPEECH LANG HEARING 1 (G0153 - U4 - GN) (Billable)	Approved	SPPS-TN-MIDDLE - SPEECH PATHOLOGY SPECIALIST, LLC-MIDDLE	04/01/2026	04/30/2026
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	G0153-U4-GN - SPEECH LANG HEARING 1 (G0153 - U4 - GN) (Billable)	Approved	SPPS-TN-MIDDLE - SPEECH PATHOLOGY SPECIALIST, LLC-MIDDLE	05/01/2026	05/31/2026
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	G0153-U4-GN - SPEECH LANG HEARING 1 (G0153 - U4 - GN) (Billable)	Approved	SPPS-TN-MIDDLE - SPEECH PATHOLOGY SPECIALIST, LLC-MIDDLE	06/01/2026	06/30/2026
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	G0153-U4-GN - SPEECH LANG HEARING 1 (G0153 - U4 - GN) (Billable)	Approved	SPPS-TN-MIDDLE - SPEECH PATHOLOGY SPECIALIST, LLC-MIDDLE	07/01/2026	07/31/2026
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	G0153-U4-GN - SPEECH LANG HEARING 1 (G0153 - U4 - GN) (Billable)	Approved	SPPS-TN-MIDDLE - SPEECH PATHOLOGY SPECIALIST, LLC-MIDDLE	08/01/2026	08/31/2026
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	G0153-U4-GN - SPEECH LANG HEARING 1 (G0153 - U4 - GN) (Billable)	Approved	SPPS-TN-MIDDLE - SPEECH PATHOLOGY SPECIALIST, LLC-MIDDLE	09/01/2026	09/30/2026

Region	Funding Source	Service(s)	Status	Service Provider	Service Date From	Service Date To
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	G0153-U4-GN - SPEECH LANG HEARING 1 (G0153 - U4 - GN) (Billable)	Approved	SPPS-TN-MIDDLE - SPEECH PATHOLOGY SPECIALIST, LLC-MIDDLE	10/01/2026	10/31/2026
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	G0153-U4-GN - SPEECH LANG HEARING 1 (G0153 - U4 - GN) (Billable)	Approved	SPPS-TN-MIDDLE - SPEECH PATHOLOGY SPECIALIST, LLC-MIDDLE	11/01/2026	11/30/2026
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	G0153-U4-GN - SPEECH LANG HEARING 1 (G0153 - U4 - GN) (Billable)	Approved	SPPS-TN-MIDDLE - SPEECH PATHOLOGY SPECIALIST, LLC-MIDDLE	12/01/2026	12/31/2026
Middle	CAC - COMPREHENSIVE AGGREGATE CAP	G0153-U4-GN - SPEECH LANG HEARING 1 (G0153 - U4 - GN) (Billable)	Approved	SPPS-TN-MIDDLE - SPEECH PATHOLOGY SPECIALIST, LLC-MIDDLE	01/01/2027	01/16/2027

Funding Source Cost by Plan Year

Funding Source	Cost
CAC - COMPREHENSIVE AGGREGATE CAP	437888.55

Funding Source Cost by Calendar Year

Funding Source	Year	Cost
CAC - COMPREHENSIVE AGGREGATE CAP	2027	10064.19
CAC - COMPREHENSIVE AGGREGATE CAP	2026	225838.15

Deleted Service Supports

Nothing found to display

Questionnaire

Jump to

TN DIDD Person Centered Support Plan

Person Centered
Support Plan >

Person Centered
Support Plan Part
2 - Employment,
Learning &
Education

Person Centered
Support Plan Part
3 - My Home
Information

Person Centered
Support Plan -
Part 4 - Me!!

Person Centered
Support Plan -
Part 5

Medication Risk
Assessment &
Planning

Referrals

Navigating my PCSP

MCO Member ID

Date of last annual
or comprehensive
review and update

11/18/2025

If not annual plan,
date of most
recent change to
my plan

If I am changing
my plan, what
prompted the
change?

Who I asked to
help write my plan

- Mary Gregory, Administrator
- Nora Lutche, Speech Language
Pathologist

Plan participation

- Staff
- Conservator
- Clinicians

My Communication Style

5/11/26, 5:20 PM	Therap :: Individual Plan	
My Preferred Language	English	
	How do I make my needs known?	
	Barbara verbally communicates her wants and needs. When Barbara is ill, she will appear to be sad. It is important for staff to pay attention to her body language. She will also communicate that she doesn't feel well. Barbara may have a blank stare or slurred speech at which time staff should check to make sure she has taken her medication and followed her PKU diet. When Barbara is in pain she will tell staff where she is hurting although she may continue to express pain even after receiving medical treatment and this may be due to attention seeking. When Barbara is happy she is smiling, talking and laughing. When upset Barbara will become quiet and it may become more difficult to understand what she is saying. It is important that staff talk with her to help her work through whatever has upset her.	
My primary (non-paid) caregiver's Preferred Language	English	
	If I and/or my primary caregiver require an interpreter, please specify language and dialect	
I like to receive my information by	Phone Call	
	If answered "Other" above, please specify	

Assistive technology / Enable Technology that I use to communicate includes	When she has the hearing aids available, Barbara will uses the hearing aids to help with communication.
I am currently working with a speech therapist	Yes
I have a communication chart	No

My LTSS Information

My cost cap / expenditure cap is	CAC
---	-----

The annual cost of my services is under my cost cap	Yes
--	-----

**Enrollment
Priority Category
for ECF Choices**

**ECF CHOICES
enrollment date or
1915(c) enrollment
date**

Frequency of support coordination contacts	Once a month
---	--------------

**If answer above
to: Frequency of
support
coordination
contacts is
"Other", please
specify**

**Do I participate in
Consumer
Direction (CD) or
Self Direction?**

No

**This year my
Coordinator and I
discussed the
option of
Consumer
Directing my
eligible services**

No

**Date of
Conversation**

11/18/2025

**This year I
received
education about
abuse, neglect
and exploitation**

Yes

**Date of
Conversation**

11/18/2025

**I have concerns
about abuse,
neglect and
exploitation**

No

**If there are
concerns with
ANE, explain
concerns and how
they will be
addressed**

None

My Decision Making & Rights

**I prefer to make
decisions**

Someone else makes decisions for me

**If previous
question was
answered "Other",
please specify**

I (and my natural supports(s)) would like information regarding decision making and rights

No

If you answered yes about information regarding decision making, what information do you wish to receive?

If previous question was answered "Other", please specify

I have an Advanced Directive

No

If you don't have an Advanced Directive, would you like to receive information about Advanced Directives?

No

If you already have information on Advanced Directives, who provided that information to you?

I have a Declaration for Mental Health Treatment

No

**I have burial plans
and preferences
I'd like to share**

According to her previous provider, Barbara does have one burial policy through United Life for \$2,000. However, there is no evidence it exists according to her conservator. The COS has agreed that in case of death, her provider agency will contact DIDD and utilize the DIDD burial program (Olive Johnson is the DIDD point of contact and can be reached at 615-532-6543). DIDD has an agreement with a funeral home for these situations that covers transportation to a cemetery; a coffin; the burial; and a headstone.

**Additional
information I'd like
to share about my
decision making
includes**

Barbara likes to be in charge of her life and likes to make her own decisions. She can make day to day decisions regarding what to wear and where she would like to go. Sometimes people must explain to Barbara why her decision may not be a good choice and she generally makes a positive choice. Barbara must be in charge of her daily routine.

Michael Dunn Center is the court appointed limited conservator for Barbara with the decision making authorities/responsibilities that are specified in the court order. In making the placement decision, Michael Dunn Center expects the agency to be responsible for the safety and security of Barbara. Unless removed by the court, the person has the right to visit with whomever they wish. Michael Dunn Center does not have authority to approve visits and community outings. Furthermore, Michael Dunn Center does not have authority to exempt the agency from Protection from Harm responsibilities.

My Community Activities

**Things I currently
do in the
community
include**

Barbara will usually choose to stay at home. When out in the community, she likes to go shopping, get her hair cut, and to get sweet tea.

Things I would like to do or learn about in the community include

My ideal volunteer activity or role includes

I would like to become more independent in the community with

Supports I need to become more independent or maintain the life I want in the community include

It is important for Barbara have access to her personal hearing device and dentures while out in the community. Barbara does not like walking down the stairs. It is important for staff to be close by to ensure she remains safe and follow her Ambulation Staff Instructions. It is important for Barbara to have access to a wheelchair when she will be ambulating long distances in the community. It is important for Barbara for staff to ensure her PKU diet is being followed as recommended.

Barbara receives community participation and the intermittent wrap around services. Barbara is able to go out in the community to places of her choice and then able to return home to take a nap, take care of her hygiene needs, or work on leisure activities. Barbara enjoys going into the community but she sometimes becomes tired, wants to return home, and rest..

Barbara needs help from staff in making purchases for herself. Staff also must ensure she has enough money and receives the correct amount of change in return. She tends to want to kiss and hug people all the time and has to be reminded that this is not appropriate to everyone. Barbara relies on staff to inform her of upcoming events/activities she might enjoy. Barbara will decide which activity/event she wants to do once she has been given her options. It is also important that Barbara is not left in a vehicle, in the community or in her home.

Barbara is transported by her day staff in a regular passenger vehicle which is important for Barbara. Barbara does not require any specialized transportation. Barbara requires staff to provide assistance to ambulate safely while in the community

Natural/Informal Supports

Jump to



Nothing found to display

Restrictions

Jump to



Does the Individual have any Restrictions? No

Action Plans

Jump to



Action Plan 1

Desired Outcome Barbara follows prescribed meal plan daily for PKU to improve her health (including skin, sleep, and mood disorders). Barbara decreases her risk of complication from GI disease by following daily guidelines for improved regularity and GERD.

Need/Issue Barbara has developed a history of PKU, GERD, Chronic Constipation, and High Cholesterol.

Measurable Steps That Will Be Taken to Reach Desired Outcome

Description of Measurable Step	Responsible Person	How Often or Due Date	Where to Record	Notes
Barbara follows the prescribed heart healthy (low sodium, low saturated fat and low cholesterol), high fiber meal plan to improve her health.	For Home: Barbara and RD For Work: For Other: Barbara and RD	Daily	Monthly notes	Staff: Avoid high PHE foods: dairy (cow's milk, yogurt), diet drinks, meat and eggs; Rx rice milk as needed and Sweet' n Low sweetener only; PKU supplements daily as ordered; Track PHE intake via daily food intake logs; VUMC RD to assess tyrosine (≥ 1.0 mg/dL) and phenylalanine (2-5 mg/dL) level Bi-annually; Rx MVI with FeSO ₄ , calcium, and vitamin D Offer at least 20-25 gm. of fiber: 5 servings of fruit and vegetables, and 3 servings of whole grains; RX prune juice, as needed; Consume at least 9+ cups of reduced sugar fluids; Provide meal plan and lifestyle changes for GERD; Maintain upright position 1 hr. after meals or per MD order; Rx appropriate activity as tolerated; Limit caffeine, citrus, as well as spicy and other highly acidic foods. RD to monitor food/fluid intakes and elimination records; all labs especially B12, calcium, and iron stores with MD.
Barbara eats safely including the total assistance as well as portion control of liquid. taking teaspoon size bites, clearing her mouth before eating more, drinking liquids every 2-3 bites and not talking during meals	For Home: Barbara and SLP For Work: For Other: Barbara and SLP	Daily	Monthly notes	Staff offers taking teaspoon size bites, clearing her mouth before eating more, drinking liquids every 2-3 bites and not talking during meals

Action Plan 2

Desired Outcome Barbara wants to communicate effectively with others

Need/Issue Barbara is hard of hearing and had lost her hearing aids.

Measurable Steps That Will Be Taken to Reach Desired Outcome

Description of Measurable Step	Responsible Person	How Often or Due Date	Where to Record	Notes
Barbara receives hearing aids.	For Home: Barbara and SLP For Work: For Other: Barbara and SLP	Daily	Monthly notes	
Barbara tells staff when hearing aids are not working.	For Home: Barbara and SLP For Work: For Other: Barbara and SLP	Daily	Daily Documentation	
Barbara completes 1-2 step instructions and verbalized her needs and concerns.	For Home: Barbara and SLP For Work: For Other: Barbara and SLP	Daily	Monthly Documentation	

Action Plan 3

Desired Outcome Barbara does much for herself to increase her independence.

Need/Issue Due to her declining health, she cannot perform previous activities such as household chores & ADLs.

Measurable Steps That Will Be Taken to Reach Desired Outcome

Description of Measurable Step	Responsible Person	How Often or Due Date	Where to Record	Notes
Barbara chooses to participate in community activities at least 1x per week .	For Home: Barbara, SL, & CP For Work: For Other: Barbara, CP & SL	Daily	Monthly Documentation	

Discussion Records

[Jump to](#)


Nothing found to display

Charting the LifeCourse

Jump to



Nothing found to display

Document Checklist

Jump to

Checklist	Attachment	Description	Uploaded By
Advance Directive			
Atypical Form			
Behavior Support Plan			
Behavioral Data			
Behavioral Health Crisis Prevention, Intervention and Stabilization Services Plan			
Budget Worksheet			
Caregiver Assessment			
Communication Chart			
Comprehensive Needs Assessment			

Conservatorship Documentation	Barbara.Phagan.3621.conservatorshippapers.06.30.2021.pdf (473.70 KB)		Jennifer Pitroski, Independent Support Coordinator
Consumer Direction Documentation			
Crisis Plan			
Declaration for Mental Health Treatment			
Dental Plan of Care			
Doctors Orders/Treatment Plans			
Employment Plans			
Enabling Technology Support Plan			
Functional Assessment			
Human Rights			
Individual Demographics Form (IDF) PDF from Therap	Barbara.Phagan.3621.IDFAttachment.02.27.2024.pdf (534.84 KB)		Jennifer Pitroski, Independent Support Coordinator
Individual Education Plan (IEP)			

LON Exception Form	Barbara.Phagan.3621.LON.10.30.2025.pdf (128.99 KB)		Jennifer Pitroski, Independent Support Coordinator
Medication List			
Medication Risk Assessment and Planning Tool			
PA/SHC Calendar			
Safety Risk Determination			
Signature Sheet	Barbara.Phagan.3621.SignatureSheet.11.18.2025.pdf (94.25 KB)		Jennifer Pitroski, Independent Support Coordinator
Sleep Data			
Support Intensity Scale (SIS) Assessment			
Therapy Treatment Plan/Plan of Care			
Transition Plan			
Vehicle/Home Modification Documentation			
Other	Barbara.Phagan.3621.PSD.10.30.2025.pdf (81.05 KB)		Jennifer Pitroski, Independent Support Coordinator

Other	Barbara.Phagan.3621.RiskTool.10.30.2025.pdf (108.36 KB)		Jennifer Pitroski, Independent Support Coordinator
Other	Barbara.Phagan.3621.InformedChoice.11.18.2025.pdf (94.30 KB)		Jennifer Pitroski, Independent Support Coordinator
Other	Barbara.Phagan.3621.SpeechPOC.12.2025.pdf (143.67 KB)		Jennifer Pitroski, Independent Support Coordinator

Participants

Jump to



Participant	Relationship with the Individual	Receive Copy of the Plan
Melissa Redmond	Guardian	Yes
Nora Lutche, Speech Language Pathologist		Yes
Maureen Udez	DSP/ Kharis	Yes
Amanda Bailey	QA/Kharis	Yes
Marq Gilchrest	ISC/BGC,Inc.	Yes
Josiah Ozm	Program Coordinator	Yes
Comfort	Kharis	Yes

Signature Log

Jump to

Acknowledgement Report

View PDFs

© 2026 Therap Services LLC. Version: 2026.5.0.1 (20260507-0930) on taapp02wI02

Patents • (<https://www.therapservices.net/patents/>) Compliance • (</ma/admin2/privacyPolicy>)

Website • (<https://www.therapservices.net/>) Help & Support • (<https://help.therapservices.net/s/>) Live Help